

Guideline

OH/C048

May 2018

Cerebral Space Occupying Lesions

1 Scope

Trustwide document

2 Purpose

Trustwide document to ensure affective assessment and treatment of all Oncology and Haematology patients.

3 Introduction

The following guidelines suggest the appropriate investigations and treatments for patients presenting with a cerebral space occupying lesion.

4 Management of Brain Tumours in Secondary Care

4.1 Imaging

All patients with a suspected primary brain tumour must have an MRI scan that as a minimum includes T1, T2 and T1 with gadolinium. DWI imaging should also be undertaken locally to reduce the risk of missing abscesses.

All patients with metastases where a Neuroscience SMDT opinion is to be sought must have an MRI scan that as a minimum includes T1, T2 and T1 with gadolinium prior to radiotherapy.

All patients with a suspicion of metastatic brain tumours must have a CXR or a CT of the chest abdomen and pelvis.

All patients with confirmed metastatic brain tumour must have a CT of the chest abdomen and pelvis. Patients with known primary disease should be restaged.

Please see the 'Imaging Recommendations for Referring Hospitals' in **Appendix 1**

4.2 Other investigations

All patients must have FBC, U&E/Creatinine/LFTs &, clotting.

Patients with suspected metastases must have relevant tumour markers and calcium.

4.3 Treatment

Except with suspected Lymphoma all patients should have Dexamethasone 8mgs bd, plus a proton pump inhibitor.

Stop Asprin and Clopidogrel (no surgery for ten days after even a single dose of either).

4.4 Steroids

Dexamethasone rapidly resolves symptoms, improves appetite and is an anti-emetic. It reduces raised ICP by minimising surrounding oedema.

Dexamethasone has a half-life of two days and therefore takes days to build up to a steady level. It disturbs sleep if given late in the day.

Dexamethasone given for more than 2-3 weeks will suppress adrenal function and therefore needs to be weaned carefully in this group of patients. In general the dexamethasone dose should be the minimum required to control symptoms.

Try to avoid dexamethasone pre-operatively in patients with suspected lymphoma.

For cases where the patient is rapidly deteriorating, low dose Dexamethasone may be given, but this should be discussed with a neurosurgeon first.

4.5 Proton pump inhibitors

A proton pump inhibitor such as Omeprazole should be given to protect the stomach lining whilst taking steroids.

In some instances, oral thrush may become apparent. Nystatin 1ml QDS or a course of Fluconazole may then be indicated as a form of treatment.

4.6 Analgesia

Paracetamol is safe and effective either alone or in combination.

Codeine has been used in Neurosurgery because it is said to be less sedative than the other opioids. However this is not proven and Tramadol or Morphine are acceptable alternatives.

Non steroidal anti-inflammatory drugs are effective analgesics and widely used in these patients.

Some surgeons are concerned about their effect on blood clotting and wound healing. In combination with steroids there is a significant risk of gastric ulceration.

4.7 Anti-epileptics

Many patients with intra-cerebral tumours develop epilepsy and have either major seizures or, more frequently, focal attacks. A change in seizure character may indicate a transformation of an existing tumour.

Routine prophylactic use of anticonvulsants is unproven but it is not unreasonable to start therapy after a single seizure given the patient has a structural abnormality and is likely to have a continuing liability to epilepsy.

Carbamazepine as a Retard preparation is probably the best initial drug and the dose should be adjusted until the blood level is within the therapeutic range. The starting dose is 100 mg bd, increasing after two weeks to 200 mg bd after which the blood level should be estimated and the dose adjusted if necessary.

If poorly tolerated then Lamotrigine and Sodium Valproate are alternatives though it should be noted that blood Valproate levels are of limited clinical value.

If compliance is a problem then Phenytoin is a suitable alternative as it can be taken once daily. The starting dose is 200 mg per day and once again, blood levels should be estimated after seven to ten days.

If seizures continue despite taking a first-line anticonvulsant, Levetiracetam 250 mg bd can be added to the regime, increasing according to response and tolerance with a maximum dose of 1.5 g bd.

5 Monitoring compliance with the effectiveness of this document

- a) Process for monitoring compliance and effectiveness: Review of incident forms, as recorded on RMIS, for non-compliance and the results presented to the Patient Safety & Governance group and relevant Standards Setting Group - the minutes of these meeting are retained for a minimum of 5 years.
- b) Standards/Key Performance Indicators: This process forms part of a quality system accredited to International Standard BS EN ISO 9001:2008. The effectiveness of the process will be monitored in accordance with the methods given in the quality manual, QM1.00

6 References

Clinical Guidelines for the Management of Neuro-Oncology
Anglia Cancer Network, version 2. Ref: AngCN-SSG-BC3

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This document complies with the Cambridge University Hospitals NHS Foundation Trust service Equality and Diversity statement.

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Appendix 1. Imaging Recommendations for Referring Clinicians

